

Transurethral resection of the prostate (TURP)

Prostate cancer may block the tube that drains urine from the bladder. An operation called a transurethral resection of the prostate (TURP) can help.

On this page

- [Why am I having a TURP?](#)
- [Having a TURP](#)
- [After your TURP](#)
- [Side effects of a TURP](#)
- [Going home after a TURP](#)
- [How we can help](#)

Why am I having a TURP?

Your surgeon may offer you a transurethral resection of the prostate (TURP) if you are having difficulty passing urine (peeing). A TURP is not done to treat [prostate cancer](#).

The [prostate gland](#) surrounds the urethra, the tube that drains urine from the bladder. If the prostate cancer starts to press on the urethra, it can restrict the flow of urine. The aim of the surgery is to remove the area of prostate that is pressing on the urethra. It can help you pass urine more easily again and improve symptoms.

You may have a TURP before [radiotherapy](#). This is because radiotherapy can make passing urine more difficult during and for a while after treatment.

Before surgery, you may have a [pre-operative assessment](#). This may involve having some tests to check your general health and fitness.

Having a TURP

A TURP can be done under a general anaesthetic (where you are asleep). Or you may have a spinal anaesthetic, which is an injection of anaesthetic around the spine. We have more information about [types of anaesthetic](#).

During a TURP, the surgeon passes a thin tube through the urethra into the prostate. This tube has a cutting instrument and tiny camera. The surgeon uses the cutting instrument to shave off the inner area of the prostate. These pieces of prostate are sent to the laboratory to be tested.

A TURP can also be done using a special laser to remove the inner area of the prostate. This is only done in some specialist treatment centres. Your surgeon will advise you if this is suitable for you.

You usually go into hospital on the day of your surgery, or the day before.

The nurse will give you compression stockings to put on. These are called TED stockings.

After your TURP

If you are having a general anaesthetic, your surgeon, anaesthetist or nurse will explain what to expect when you wake up after surgery.

Immediately after surgery, the nurses can help you with washing and going to the toilet. But they will encourage you to be up and moving around as soon as possible. They can help you move safely with your catheter. You may feel anxious about this, but moving around prevents complications and helps your recovery.

You will usually have a drip into a vein in your arm (intravenous infusion). This will stay in for a few hours after your operation, until you are eating and drinking again.

You can usually go home 3 or 4 days after a TURP. We have more information about what to expect [after surgery](#).

Urinary catheter

During surgery, you will have a tube put in your urethra. This tube drains urine from your bladder. It is called a catheter. The catheter keeps your bladder empty while the prostate and urethra heal. Try to drink plenty of fluids to help keep the catheter draining well.

For a while after a TURP, your urine will have blood in it. To stop blood clots blocking the catheter, fluid is passed into the bladder and drained out through the catheter. This is called bladder irrigation. It helps to wash out the bladder. The blood will slowly clear from your urine and then the catheter can be taken out.

You may still get the feeling that you want to pass urine (pee) when the catheter is in. This is called a bladder spasm. Sometimes you leak urine around the catheter when this happens. These spasms are normal but can be uncomfortable. If they last a long time and happen often, tell your nurses on the ward.

You may need the catheter for a while after you go home. This is because swelling caused by the surgery may make it difficult for you to pass urine.

The nurses on the ward will show you how to look after your catheter before you go home. They will give you spare bags to take home. If you need more support, they can arrange for a district or community nurse to visit you at home.

Removing the catheter

When the catheter is first removed, you may find it difficult to pass urine. This will improve. You may see blood in your urine for 24 to 48 hours, but it should start to clear. There may be some old bits of dried blood clot after 7 to 10 days. Drinking plenty of fluids can help.

If you cannot pass urine after your catheter has been removed, call the number you have been given straight away or go straight to the emergency department (A&E) at the hospital. You may need another catheter for a couple of days before it can be removed again.

You may leak urine after a TURP. This is called urinary incontinence. It usually improves within a few weeks. You may need to wear a pad until it improves. You may notice that you want to pass urine more urgently or frequently for a while too.

Preventing blood clots

After your surgery, the nurses will encourage you to get up and move around. This is to help prevent complications such as [blood clots](#) in the veins in the legs. You will continue to wear your compression stockings (TED stocking) to help prevent clots. Your nurse can explain how to wash and care for them. They may give you a spare pair to take home. They can advise you when you no longer need to wear the stockings

Side effects of a TURP

There are some side effects after having a TURP. Some side effects improve over time, but some are permanent.

Pain

You may have pain and discomfort for a few days after your operation. You will be given [painkillers](#) to help with this. In hospital, you may have painkillers through a drip (infusion) in your vein, or by injection. This will then be replaced with painkiller tablets. Tell the nurses on the ward if you are still in pain. They will give you tablets to take home with you and explain how to take them.

If at any time you have burning or stinging when you pass urine tell your cancer team. They may check to see if you have a urine infection.

Ejaculating into the bladder

Ejaculating into the bladder is called retrograde ejaculation. It is caused by damage to the nerves or muscles that surround the bladder neck. This is where the urethra joins the bladder. During retrograde ejaculation, semen goes backwards into the bladder instead of through the urethra in the normal way. After retrograde ejaculation, your urine may look cloudy. This is because there is semen in the bladder. It is harmless.

Erection problems

You may have problems getting an erection after the TURP. This is called [erectile dysfunction \(ED\)](#). It might depend on whether you had problems getting an erection before surgery. Your surgeon can tell you more about your risk of erection problems. Your surgeon or specialist nurse can also give you information and support.

Difficulties passing urine

Sometimes TURP can cause long term difficulties with passing urine. Talk to your urologist or specialist nurse if you are having problems. They can offer help and support and may arrange some tests.

Going home after a TURP

You can usually go home 3 or 4 days after a TURP.

Before you leave hospital, your hospital doctor will send a letter called a discharge summary to your GP. The letter is usually sent electronically so that your GP gets it within 24 hours of your discharge. You are usually given a copy to take home with you. It will explain what type of surgery you had, any medication you are taking and about your follow-up care.

If you need a district or community nurse, the ward nurses will organise this before you go home. You should also be given 24-hour telephone numbers for your hospital team in case you need to contact them.

Your surgeon, specialist nurse or nurses on the ward can give you advice on when you can start doing things such as driving or returning to work. They can also give you advice on when you can have sex again. We have more information about [sex and prostate cancer](#).

Getting support

Macmillan is also here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line for free on [0808 808 00 00](tel:0808 808 00 00).
- [Chat to our specialists online](#).
- Visit our [prostate cancer forum](#) to connect with people who have been affected by prostate cancer, share your experience, and ask your questions.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our prostate cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

National Institute for Health and Care Excellence (NICE). Prostate cancer: diagnosis and management. NICE Guideline [NG131]. Published: 09 May 2019. Last updated: 15 December 2021. Available from: www.nice.org.uk/guidance/ng131 [accessed March 2024].

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Date reviewed

Reviewed: 01 October 2025 | Next review: 01 October 2028

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help